

Access guide — ulms-highgrade-pulm-mets-er-pr-neg-q qvt

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	2024 operative report retrieval (morcellation question)	other	Standard of care	Medical-records request; no regulatory gate.	Submit a records request to the 2024 surgical facility for the full operative report and original pathology report.
2	Altitude-adjusted baseline diagnostics	other	Standard of care	Routine clinical diagnostics; no regulatory gate.	Book the echocardiogram first; LVEF is mandatory before any doxorubicin dose and is the likeliest scheduling bottleneck.
3	Pazopanib	small_molecule	Standard of care	FDA-approved (Votrient) for advanced soft tissue sarcoma after prior chemotherapy; not indicated first-line.	Nothing to do now; note it as the default approved second-line comparator when weighing trial enrollment later.
4	Pulmonary metastasectomy	other	Standard of care	Surgical procedure; no regulatory gate.	Complete PFTs with DLCO and oximetry at residence altitude now; they serve both the chemotherapy baseline and any future operability assessment.
5	SBRT to pulmonary metastases	other	Standard of care	Standard radiotherapy technique; no regulatory gate.	Nothing until response assessment; then have radiation oncology review lesion count and geometry against SBRT criteria.
6	Trabectedin monotherapy	small_molecule	Standard of care	FDA-approved (2015) for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior regimen.	No separate access work; this arrives bundled inside the LMS-04 plan as the maintenance drug.

7	Abemaciclib + gemcitabine	small_molecule	Off-label use + Clinical trial	Abemaciclib (Verzenio, Lilly) FDA-approved in breast cancer; gemcitabine generic. The sarcoma combination is investigational.	Order RB1 IHC on the archival block now; it costs one slide and decides between this trial and REC-617.
8	Afamitresgene autoleucel (MAGE-A4 TCR-T)	other	Off-label use	FDA accelerated approval (August 2024) for advanced synovial sarcoma, HLA-A*02-positive and after prior chemotherapy. Current US label holder per DailyMed: USWM CT, LLC.	Do not pursue; route the cell-therapy interest through the IMA203 PRAME basket instead.
9	All-trans retinoic acid + cemiplimab	other	Off-label use + Clinical trial	Tretinoin FDA-approved (APL) and generic; cemiplimab (Libtayo, Regeneron) FDA-approved in CSCC, BCC, NSCLC. The combination in sarcoma is investigational.	File as a post-LMS-04 option requiring no biomarker work; nothing to do now except know it exists.
10	Doxorubicin + ifosfamide (intensified)	small_molecule	Off-label use	Both components FDA-approved and generic; combination use in soft tissue sarcoma is off-label.	No access action needed; the clinical question (whether it ever beats the planned LMS-04 backbone for her) belongs to the board, not to an access pathway.
11	Doxorubicin + trabectedin, then trabectedin maintenance (LMS-04)	small_molecule	Off-label use	Both components FDA-approved: doxorubicin (generic, long-approved); trabectedin (Yondelis, FDA 2015) for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior regimen. The first-line combination is off-label relative to that label.	Complete the gating diagnostics before cycle 1, all routine and orderable locally: echocardiogram with LVEF, hepatic and renal panels with CK, CBC with reticulocytes and ferritin read against altitude-adjusted references, PFTs with DLCO and oximetry performed at her residence altitude, a documented ECOG score, and RECIST baseline imaging.

12	Eribulin	small_molecule	Off-label use	FDA-approved (Halaven) for unresectable or metastatic liposarcoma after an regimen; LMS use is off-label.	Revisit at the third-line decision point; no earlier action is available or useful.
13	Gemcitabine + docetaxel	small_molecule	Off-label use	Both components FDA-approved for other indications and available as generics; no sarcoma indication on either label.	Hold in reserve pending the baseline LVEF and PFT results that gate doxorubicin.
14	Larotrectinib (TRK inhibition, NTRK fusion-gated)	small_molecule	Off-label use + Clinical trial	Larotrectinib (Vitrakvi, Bayer): FDA tumor-agnostic approval for NTRK fusion-positive solid tumors. Repotrectinib (Augtyro, Bristol Myers Squibb): FDA accelerated approval June 13, 2024 for NTRK fusion-positive solid tumors, adult and pediatric ≥12. Fusion-negative or untested use has no basis.	Confirm the comprehensive panel order is RNA-capable for fusion detection (the Caris exome-plus-transcriptome order already queued covers this; FoundationOne CDx alone would not).
15	Letrozole	small_molecule	Off-label use	FDA-approved (breast cancer indications); generic. uLMS use is off-label.	Only revisit if the pending specimen-provenance check overturns the ER/PR result, which would trigger re-screening of the whole endocrine class.
16	Lurbinectedin + doxorubicin	small_molecule	Off-label use	Lurbinectedin (Zepzelca, Jazz/PharmaMar) FDA-approved for metastatic SCLC; doxorubicin generic. The LMS combination is investigational.	Log SaLuDo (NCT06088290) as a watch item alongside the olaparib randomized trial; its result could reshape her first-line class at relapse or inform re-treatment decisions.

17	Olaparib + temozolomide	small_molecule	Off-label use	Olaparib FDA-approved in BRCA/HRR-defined indications (ovarian, breast, pancreatic, prostate); temozolomide FDA-approved (glioma) and generic. The uLMS combination is off-label for both.	Send the somatic HRR panel and the germline hereditary-cancer panel now, off the 2024 archival block and a blood draw; results gate how hard this branch gets pushed.
18	Pegylated liposomal doxorubicin (maintenance)	small_molecule	Off-label use	FDA-approved (ovarian cancer, Kaposi sarcoma, multiple myeloma) and generic; STS maintenance use is off-label and investigational.	No action; the LMS-04 design already assigns her maintenance drug. Revisit only if trabectedin maintenance proves intolerable.
19	Pembrolizumab	monoclonal_antibody	Off-label use	FDA-approved, including tumor-agnostic indications for TMB-H (≥ 10 mut/Mb) and MSI-H/dMMR solid tumors after prior therapy. uLMS use without one of those biomarkers is off-label.	Close the cheap gates: MMR four-stain IHC plus TMB and MSI from the comprehensive genomic panel already queued; specify a platform with validated TMB reporting.
20	Trabectedin + olaparib (preclinical combination)	small_molecule	Off-label use	Each component FDA-approved separately; the combination has no clinical development program in LMS.	Treat as a research idea, not an access pathway. If a protocol testing PARP-plus-trabectedin ever opens in sarcoma, the trial screener should catch it on a re-run.
21	Trastuzumab deruxtecan (T-DXd)	ADC	Off-label use	FDA-approved (Enhertu, Daiichi including tumor-agnostic accelerated approval for HER2 IHC 3+ solid tumors after prior systemic therapy.	Add HER2 IHC to the planned stain bundle on the archival block; a 3+ result changes the option set, anything less closes the row.
22	ATR inhibition (ATRX/ALT-directed hypothesis)	small_molecule	Clinical trial	No ATR inhibitor holds regulatory approval; class-wide investigational.	Run the ATRX IHC in the planned stain bundle anyway; it is cheap, ALT-concordant in uLMS, and informs biology beyond this one hypothesis.

23	Anlotinib (catequentinib)	small_molecule	Clinical trial + Not yet accessible	Approved in China (NMPA) for advanced soft tissue sarcoma settings; investigational in the US, no FDA filing approved.	Treat as low priority; if the TKI class is wanted, pazopanib is on label and zanzalintinib has an open LMS cohort.
24	B7-H3 ADCs (MGC026;	ADC	Clinical trial	Both agents investigational; no approvals. The class's prior entrant vobramitamab duocarmazine was halted after safety findings.	Add B7-H3/CD276 IHC to the planned stain bundle; a strong positive is the single most likely surface-target finding in this histology.
25	Brenetafusp (PRAME x CD3 ImmTAC)	bispecific_other	Clinical trial	Investigational (Immunocore); phase 3 in melanoma, no approval.	The HLA typing drawn for the IMA203 row covers this class too; no additional test needed.
26	FAP-targeted radioligands ([225Ac]RTX-2358;	radioligand	Clinical trial	Both investigational; no approvals. RTX-2358 is a Ratio Therapeutics alpha-therapy program, FAP-2286 a Novartis beta-emitter theranostic.	No FAP testing needed in advance; the screening PET is the gate and the trial provides it.
27	IMA203 / IMA203CD8 (PRAME TCR-T)	other	Clinical trial	Investigational (Immatics); phase 3 development in melanoma, no approval.	Draw HLA class I allele-level typing now, on the same visit as the germline panel; it is a one-time result that gates this entire class.
28	Ivonescimab (PD-1 x VEGF bispecific)	bispecific_other	Clinical trial	Investigational in the US; no FDA approval.	Hold until the LMS-04 outcome declares itself; she remains eligible with 1-3 prior lines.
29	Pulmonary suffusion + metastasectomy	other	Clinical trial	Investigational surgical-regional technique under a single-center IND protocol.	Park until a post-induction scan shows metastasectomy-appropriate anatomy.
30	REC-617 (selective CDK7 inhibitor)	small_molecule	Clinical trial	Investigational; no regulatory approval or filing. Phase 1 dose finding.	Same first move as the abemaciclib row: RB1 IHC on the archival block.
31	Zanzalintinib (XL092)	small_molecule	Clinical trial	Investigational (Exelixis); no approval in any indication.	Park until the third-line decision point.

32	Doxorubicin + olaratumab	monoclonal_antibody	Unavailable	Approval withdrawn; olaratumab is no longer marketed following the negative ANNOUNCE confirmatory trial.	None; recorded so the answer to 'could we add olaratumab' is visibly no rather than silently missing.
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Standard of care (6)

1. 2024 operative report retrieval (morcellation question)

Aliases: operative report, morcellation

Modality: other · **Regulatory:** Medical-records request; no regulatory gate. · **Geographic scope:** Wherever the 2024 surgery took place · **Verified:** 2026-08-12

Not a treatment; this is the cheapest decisive document in the case. Whether the September 2024 hysterectomy used morcellation roughly triples recurrence risk in the published cohorts and shifts failures toward abdominopelvic dissemination, which changes surveillance imaging rather than just history-taking. Retrieval is a routine medical-records request to the 2024 surgical facility, free and requiring no one's permission but hers.

Next steps

1. Submit a records request to the 2024 surgical facility for the full operative report and original pathology report.
2. If morcellation is documented, ask the sarcoma team to add dedicated abdominopelvic surveillance to the imaging plan.
3. File the report with the reference-pathology review so the whole diagnostic timeline sits in one place.

Payer / coverage: No cost beyond possible nominal copying fees; HIPAA right of access applies.

2. Altitude-adjusted baseline diagnostics

Aliases: LVEF, echocardiogram, PFTs, DLCO, oximetry, CBC, ECOG documentation, RECIST baseline

Modality: other · **Regulatory:** Routine clinical diagnostics; no regulatory gate. · **Geographic scope:** Local to her residence; deliberately not referral-center work · **Verified:** 2026-08-12

Every gating test here is routine and orderable close to home: echocardiogram with LVEF before doxorubicin, PFTs with DLCO plus resting and exertional oximetry, CBC with reticulocytes and ferritin, hepatic and renal panels with CK before trabectedin, a formally documented ECOG, and RECIST baseline imaging before cycle 1. The one trap is geographic: her oximetry, hemoglobin and dyspnea baselines only mean something if measured at her 8,500 ft residence altitude, so these tests should be done locally rather than at a distant referral center at low elevation.

Next steps

1. Book the echocardiogram first; LVEF is mandatory before any doxorubicin dose and is the likeliest scheduling bottleneck.
2. Do PFTs with DLCO and pulse oximetry at a facility at her residence altitude, and record the altitude on the report.
3. Draw CBC with reticulocytes and ferritin now so chemotherapy-era anemia calls have an altitude-adjusted reference point.
4. Have the oncologist document an examined ECOG score; the 1 on file is an intake assumption that several trial ceilings depend on.
5. Complete RECIST baseline imaging before cycle 1, with target-lesion selection mindful of the infiltrative pattern.

Payer / coverage: All standard pre-chemotherapy workup, covered without dispute; no prior authorization issues expected for any item.

Notes: The molecular workup runs in parallel and is covered in the rows it gates: comprehensive DNA+RNA panel (Caris-style exome plus transcriptome, closing NTRK), MMR IHC and TMB (pembrolizumab row), HRR somatic and germline plus the RAD51 caveat (olaparib row), HLA typing (IMA203 row), RB1 IHC (abemaciclib and REC-617 rows), B7-H3 and HER2 IHC (ADC rows), ATRX IHC (ATR row), PD-L1 with CD3/CD8 on the lung core.

3. Pazopanib

Aliases: pazopanib, GW786034, Votrient

Modality: small_molecule · **Regulatory:** FDA-approved (Votrient) for advanced soft tissue sarcoma after prior chemotherapy; not indicated first-line. · **Guidelines:** NCCN-listed subsequent-line option for non-adipocytic STS · **Geographic scope:** US-wide · **Verified:** 2026-08-12

FDA-approved for advanced soft tissue sarcoma after prior chemotherapy, so it becomes label-eligible for her the day LMS-04 stops working, and coverage at that point is routine. PALETTE required prior anthracycline, which she will have. It is the approved later-line backbone the investigational options in this dossier have to beat.

Next steps

1. Nothing to do now; note it as the default approved second-line comparator when weighing trial enrollment later.
2. Blood pressure monitoring plan at initiation, given the class hypertension signal.

Manufacturer / sponsor contact

- **Company:** Novartis Pharmaceuticals Corporation
- **Med info phone:** 1-888-669-6682
- **Notes:** Phone verbatim from the current FDA label ('contact Novartis Pharmaceuticals Corporation at 1-888-669-6682').

Payer / coverage: On-label once she is post-chemotherapy; oral TKI, so it runs through pharmacy benefit with the usual specialty-pharmacy prior authorization rather than medical benefit.

4. Pulmonary metastasectomy

Aliases: pulmonary metastasectomy, lung metastasectomy

Modality: other · **Regulatory:** Surgical procedure; no regulatory gate. · **Guidelines:** Guideline-recognized for resectable oligometastatic sarcoma at experienced centers · **Geographic scope:** US-wide at sarcoma-capable thoracic surgery programs · **Verified:** 2026-08-12

A standard surgical service at any sarcoma-capable thoracic program, requiring a referral rather than any special access machinery; her oncologist has explicitly kept it on the table for after systemic control. The obstacles are anatomical and physiological, not logistical: bilateral infiltrative disease may never become resectable anatomy, and surgical fitness will be judged on pulmonary reserve that must be measured against her 8,500 ft baseline, with PFTs still undone.

Next steps

1. Complete PFTs with DLCO and oximetry at residence altitude now; they serve both the chemotherapy baseline and any future operability assessment.
2. Revisit resectability at the first post-induction response scan with a high-volume sarcoma thoracic surgeon.
3. Keep the infiltrative-pattern caveat explicit in that conversation; the favorable metastasectomy literature describes discrete, countable lesions.

Payer / coverage: Covered as standard surgical care when a thoracic surgeon documents resectability and fitness.

5. SBRT to pulmonary metastases

Aliases: SBRT, stereotactic body radiotherapy

Modality: other · **Regulatory:** Standard radiotherapy technique; no regulatory gate. · **Guidelines:** Guideline-recognized local-therapy option for oligometastatic disease · **Geographic scope:** US-wide · **Verified:** 2026-08-12

Routinely available at any radiation oncology department with stereotactic capability; no access barrier exists beyond a referral. Whether her lesions ever become SBRT-appropriate targets is the real question, since the published series treat up to about four discrete lesions and her current pattern is bilateral and infiltrative. If induction converts the disease to oligometastatic anatomy, this is the non-surgical local option, with local control above 90% in sarcoma series.

Next steps

1. Nothing until response assessment; then have radiation oncology review lesion count and geometry against SBRT criteria.
2. Baseline PFTs matter here too; pulmonary reserve at altitude bounds how much lung can be treated.

Payer / coverage: Covered as standard radiotherapy with documentation of lesion number and intent; prior authorization is routine paperwork at any center that does this weekly.

6. Trabectedin monotherapy

Aliases: trabectedin, ET-743, Yondelis

Modality: small_molecule · **Regulatory:** FDA-approved (2015) for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior anthracycline-containing regimen. · **Guidelines:** Secondary summaries report NCCN treats single-agent trabectedin as a preferred later-line regimen for LMS · **Geographic scope:** US-wide · **Verified:** 2026-08-12

On-label and routinely covered once she has anthracycline exposure, which the planned LMS-04 induction provides; the maintenance phase of that regimen is effectively this drug continued. Today, before any anthracycline, standalone trabectedin would be off-label, but that sequencing question resolves itself the day induction starts. Access is a prescription plus central line, not a fight.

Next steps

1. No separate access work; this arrives bundled inside the LMS-04 plan as the maintenance drug.
2. Keep hepatic panels and CK on the recurring lab schedule through maintenance.

Manufacturer / sponsor contact

- **Company:** Janssen Products, LP (Johnson & Johnson)
- **Med info phone:** 1-800-526-7736
- **Product info:** <https://www.yondelis.com>

Payer / coverage: On-label after anthracycline, so coverage is standard. The safety dossier her team will monitor against (transaminitis, cytopenias, rare rhabdomyolysis) comes from SAR-3007.

Off-label use (15)

7. Abemaciclib + gemcitabine

Aliases: abemaciclib, LY2835219, Verzenio, gemcitabine

Modality: small_molecule · **Regulatory:** Abemaciclib (Verzenio, Lilly) FDA-approved in breast cancer; gemcitabine generic. The sarcoma combination is investigational. · **Geographic scope:** US: single site (Houston, TX) · **Verified:** 2026-08-12

The live trial is the point: the NCI-sponsored phase 1/2 at MD Anderson is recruiting now, its phase 2 LMS cohort accepts systemic-treatment-naïve patients, and prior doxorubicin or trabectedin would not exclude her later, so this door stands open both before and after LMS-04. Entry is gated on a single Rb-intact IHC stain she has not had. Both drugs are separately marketed, so off-label use is possible in principle, but the preclinical antagonism data for concurrent dosing in Rb-intact tumors argues for doing this inside the protocol or not at all.

Next steps

1. Order RB1 IHC on the archival block now; it costs one slide and decides between this trial and REC-617.

2. If Rb-intact, call the MD Anderson site line (866-632-6789) to ask about phase 2 LMS cohort slots and whether treatment-naïve entry or post-LMS-04 entry fits the current cohort.
3. Confirm her actual ECOG before any screening call; the assumed 1 has never been documented.
4. Weigh the Houston travel burden explicitly with her; prefers_trials is an assumed default, not a stated preference.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06498248	1/2	recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Eli Lilly and Company (abemaciclib)
- **Med info phone:** 1-800-545-5979
- **Notes:** Label lists 1-800-LillyRx (1-800-545-5979), verbatim from the current Verzenio FDA label.

Payer / coverage: Off-label combination coverage outside the trial would be a hard sell with no published sarcoma efficacy; the trial supplies abemaciclib free. If ever pursued off-trial, expect specialty-pharmacy prior authorization for abemaciclib and an uphill appeal.

8. Afamitresgene autoleucel (MAGE-A4 TCR-T)

Aliases: afamitresgene autoleucel, afami-cel, ADP-A2M4, Tecelra

Modality: other · **Regulatory:** FDA accelerated approval (August 2024) for advanced synovial sarcoma, HLA-A*02-positive and MAGE-A4-expressing, after prior chemotherapy. Current US label holder per DailyMed: USWM CT, LLC. · **Geographic scope:** US authorized treatment centers; label-restricted to synovial sarcoma · **Verified:** 2026-08-12

FDA-approved, which technically puts an off-label path on the books, but the honest answer is that she cannot get it: the accelerated approval covers advanced synovial sarcoma in HLA-A*02-positive, MAGE-A4-positive patients, the product is manufactured per patient at authorized treatment centers under that label, and no manufacturer builds an autologous TCR-T for an off-label histology. No MAGE-A4 program currently enrolls LMS either. The row anchors what the engineered-T-cell class can do in sarcoma; her reachable version of it is the PRAME row.

Next steps

1. Do not pursue; route the cell-therapy interest through the IMA203 PRAME basket instead.
2. The HLA typing already recommended would also be the first gate if a MAGE-A4 program ever opens to LMS.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04024768	2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** USWM CT, LLC (US WorldMeds); developed by Adaptimmune
- **Med info phone:** 1-855-246-9232
- **Notes:** Phone verbatim from the current Tecelra FDA label ('contact USWM CT, LLC at 1-855-246-9232').

Payer / coverage: Coverage exists only inside the label; no payer or treatment center will support off-label manufacture for uLMS.

9. All-trans retinoic acid + cemiplimab

Aliases: all-trans retinoic acid, ATRA, tretinoin, cemiplimab, REGN2810, Libtayo

Modality: other · **Regulatory:** Tretinoin FDA-approved (APL) and generic; cemiplimab (Libtayo, Regeneron) FDA-approved in CSCC, BCC, NSCLC. The combination in sarcoma is investigational. · **Geographic scope:** US: single site (Columbus, OH) · **Verified:** 2026-08-12

The Ohio State single-site phase 2 is recruiting and takes LMS after standard chemotherapy with no biomarker gate, which makes it one of the few immunotherapy routes here that does not wait on her missing profiling; it opens once LMS-04 fails. Both components are marketed (tretinoin generic, cemiplimab approved in skin and lung cancers), so off-label use is possible on paper, but there is no published sarcoma efficacy to justify it outside the protocol.

Next steps

1. File as a post-LMS-04 option requiring no biomarker work; nothing to do now except know it exists.
2. At progression, email OSUCCCClinicaltrials@osumc.edu or call 800-293-5066 to confirm slot availability and screening timelines.
3. Check RECIST-measurable disease on the baseline imaging with this trial's requirement in mind.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06528769		recruiting	yes	The Ohio State University Comprehensive Cancer Center; OSUCCCClinicaltrials@osumc.edu; 800-293-5066

Manufacturer / sponsor contact

- **Company:** Regeneron Pharmaceuticals (cemiplimab); tretinoin multi-source generic
- **Med info phone:** 1-877-542-8296
- **Notes:** Phone verbatim from the current Libtayo FDA label ('contact Regeneron at 1-877-542-8296').

Payer / coverage: Off-trial, cemiplimab in sarcoma would be an unsupported off-label request; inside the trial, drug is supplied. No biomarker prerequisite means no testing spend to keep this option alive.

10. Doxorubicin + ifosfamide (intensified)

Aliases: doxorubicin, Adriamycin, ifosfamide, Ifex, mesna

Modality: small_molecule · **Regulatory:** Both components FDA-approved and generic; combination use in soft tissue sarcoma is off-label. · **Geographic scope:** US-wide · **Verified:** 2026-08-12

Both agents are generic and universally available; ifosfamide has no sarcoma-specific US indication, so the pairing is off-label but routine at sarcoma centers. EORTC 62012 showed more response and toxicity without an overall survival win, and LMS-04 has largely displaced it for this histology. It stays on the list as an available combination, not a recommended one for her.

Next steps

1. No access action needed; the clinical question (whether it ever beats the planned LMS-04 backbone for her) belongs to the board, not to an access pathway.

Payer / coverage: Generic; requires mesna co-administration and growth-factor support, none of which raises coverage problems.

11. Doxorubicin + trabectedin, then trabectedin maintenance (LMS-04)

Aliases: doxorubicin, Adriamycin, trabectedin, ET-743, Yondelis

Modality: small_molecule · **Regulatory:** Both components FDA-approved: doxorubicin (generic, long-approved); trabectedin (Yondelis, FDA 2015) for unresectable or metastatic liposarcoma or leiomyosarcoma after a prior anthracycline-containing regimen. The first-line combination is off-label relative to that label. · **Guidelines:** Secondary summaries report NCCN lists doxorubicin + trabectedin among first-line options for LMS post-LMS-04; category not independently verified this run · **Geographic scope:** US-wide; any sarcoma-capable infusion center · **Verified:** 2026-08-12

Both drugs are FDA approved and any sarcoma infusion center can give them, but the combination she is about to start sits outside the trabectedin label: Yondelis is approved for liposarcoma or leiomyosarcoma after a prior anthracycline-containing regimen (verified against the current label, revised 12/2025), and she is anthracycline-naïve. First-line use with doxorubicin is therefore off-label prescribing carried by the LMS-04 phase 3 data, not by the label. The realistic obstacle is trabectedin prior authorization, and the time-critical items before cycle 1 are diagnostic, not access.

Next steps

1. Complete the gating diagnostics before cycle 1, all routine and orderable locally: echocardiogram with LVEF, hepatic and renal panels with CK, CBC with reticulocytes and ferritin read against altitude-adjusted references, PFTs with DLCO and oximetry performed at her residence altitude, a documented ECOG score, and RECIST baseline imaging.
2. Request the September 2024 operative report to settle whether morcellation occurred; it changes surveillance imaging, and it costs nothing but a records request.
3. Submit the trabectedin prior authorization as an off-label first-line request citing LMS-04 (PMID 35835135 and PMID 39231341) rather than waiting for a denial.

4. Arrange central venous access; trabectedin runs as a 24-hour infusion through a central line.
5. For label or access questions, call Janssen medical information at 1-800-526-7736.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02937358		completed	yes	—

Manufacturer / sponsor contact

- **Company:** Janssen Products, LP (Johnson & Johnson); trabectedin originator PharmaMar
- **Med info phone:** 1-800-526-7736
- **Product info:** <https://www.yondelis.com>
- **Notes:** Phone taken verbatim from the current FDA label ('For more information, call 1-800-526-7736 or go to www.YONDELIS.com'). The label also directs patients to J&J withMe support resources.

Payer / coverage: Doxorubicin is generic and uncontroversial. Trabectedin is the expensive component and its label requires prior anthracycline, so the prior authorization should be built as an off-label first-line request from the start: cite the LMS-04 primary report (Lancet Oncol 2022, PMID 35835135), the final overall survival analysis (NEJM 2024, PMID 39231341), and whatever NCCN compendium listing the treating center's pharmacy can pull. Centers that treat sarcoma routinely win this appeal; the case should not be filed as if it were on-label.

Notes: The single most decision-relevant access finding in this case: the planned regimen is deliverable everywhere, but it is off-label at this line and the payer file should be assembled that way from day one.

12. Eribulin

Aliases: eribulin, E7389, Halaven

Modality: small_molecule · **Regulatory:** FDA-approved (Halaven) for unresectable or metastatic liposarcoma after an anthracycline-containing regimen; LMS use is off-label. · **Guidelines:** NCCN-listed later-line option for STS · **Geographic scope:** US-wide · **Verified:** 2026-08-12

Marketed and easy to obtain, but the FDA sarcoma indication is liposarcoma only; in her leiomyosarcoma it is off-label with a neutral LMS subgroup result behind it (OS HR 0.93 vs dacarbazine). The label also requires two prior regimens including an anthracycline, so this door is two lines away regardless. Payers sometimes push back on LMS use; compendium listings usually settle it.

Next steps

1. Revisit at the third-line decision point; no earlier action is available or useful.

Manufacturer / sponsor contact

- **Company:** Eisai Inc.
- **Med info phone:** 1-877-873-4724
- **Product info:** <https://www.halaven.com>

- **Notes:** Phone verbatim from the current FDA label ('contact Eisai Inc. at (1-877-873-4724)').

Payer / coverage: Off-label for LMS; if a payer denies, the appeal rests on compendium listing plus the Study 309 whole-trial OS benefit, while acknowledging the LMS subgroup was neutral.

13. Gemcitabine + docetaxel

Aliases: gemcitabine, Gemzar, docetaxel, Taxotere

Modality: small_molecule · **Regulatory:** Both components FDA-approved for other indications and available as generics; no sarcoma indication on either label. · **Guidelines:** Listed in NCCN soft tissue sarcoma guidelines as an option for LMS (long-established) · **Geographic scope:** US-wide · **Verified:** 2026-08-12

Both drugs are generic, cheap, and stocked at every infusion center; neither carries a sarcoma indication, so use in uLMS is long-standing off-label practice that payers do not fight. Its role here is the first-line fallback if the baseline echocardiogram or her pulmonary reserve at 8,500 ft rules out full-dose doxorubicin. No access work is needed beyond a routine order.

Next steps

1. Hold in reserve pending the baseline LVEF and PFT results that gate doxorubicin.
2. If used, plan G-CSF support per the GOG dosing experience.

Payer / coverage: Generic pricing keeps prior authorization friction minimal; compendium support for sarcoma use is long established.

14. Larotrectinib (TRK inhibition, NTRK fusion-gated)

Aliases: larotrectinib, LOXO-101, Vitrakvi, repotrectinib, Augtyro, entrectinib, Rozlytrek

Modality: small_molecule · **Regulatory:** Larotrectinib (Vitrakvi, Bayer): FDA tumor-agnostic approval for NTRK fusion-positive solid tumors. Repotrectinib (Augtyro, Bristol Myers Squibb): FDA accelerated approval June 13, 2024 for NTRK fusion-positive solid tumors, adult and pediatric ≥ 12 . Fusion-negative or untested use has no basis. · **Geographic scope:** US-wide · **Verified:** 2026-08-12

This branch is gated on a test, not on supply: larotrectinib and repotrectinib both hold tumor-agnostic FDA approvals for NTRK fusion-positive solid tumors, so a positive fusion result makes an oral TRK inhibitor label-available rather than off-label. No fusion assay of any kind has been run on her tumor, and a DNA-only panel like FoundationOne CDx can miss NTRK fusions; the ordered panel must be RNA-capable. The repotrectinib basket TRIDENT-1 is also still recruiting across 49 US sites if trial access ever suits better.

Next steps

1. Confirm the comprehensive panel order is RNA-capable for fusion detection (the Caris exome-plus-transcriptome order already queued covers this; FoundationOne CDx alone would not).
2. If a fusion returns, choose between label-available larotrectinib or repotrectinib and TRIDENT-1 enrollment;

call the BMS contact center at 855-907-3286 if the trial route is preferred.

3. Treat as a cheap-to-close long shot: prevalence in true uLMS is very low, but her spindle-cell morphology with an earlier benign sign-out is exactly where a fusion-driven mimic hides.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03091216	1	recruiting	unconfirmed	BMS Clinical Trials Contact Center www.BMSClinicalTrials.com; Clinical.Trials@bms.com; 855-907-3286

Manufacturer / sponsor contact

- **Company:** Bayer HealthCare Pharmaceuticals (larotrectinib)
- **Med info phone:** 1-888-842-2937
- **Notes:** Phone verbatim from the current Vitrekvi FDA label. For repotrectinib, the Augtyro label lists Bristol-Myers Squibb at 1-800-721-5072.

Payer / coverage: With a documented NTRK fusion, tumor-agnostic coverage is routine. The entire payer risk sits upstream, in ordering a fusion-capable assay: an RNA-based or fusion-calling panel, not a DNA-only test that would return a silent false negative.

15. Letrozole

Aliases: letrozole, Femara

Modality: small_molecule · **Regulatory:** FDA-approved (breast cancer indications); generic. uLMS use is off-label. · **Geographic scope:** US-wide · **Verified:** 2026-08-12

Generic, pennies a day, and available at any pharmacy; access was never the issue. Her tumor is ER and PR under 1%, and the durable letrozole benefit in uLMS was confined to tumors expressing both receptors in more than 90% of cells, so the biology closes this door regardless of how easy the prescription is. The one recruiting letrozole trial in uLMS excludes her twice over, on receptor status and on stage.

Next steps

1. Only revisit if the pending specimen-provenance check overturns the ER/PR result, which would trigger re-screening of the whole endocrine class.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05629956	1	recruiting	no	Sarin Chhab; schhab@gog.org; 2158540770

Payer / coverage: Not applicable in practice; the receptor result forecloses use before coverage ever comes up.

16. Lurbinectedin + doxorubicin

Aliases: lurbinectedin, PM01183, Zepzelca, doxorubicin

Modality: small_molecule · **Regulatory:** Lurbinectedin (Zepzelca, Jazz/PharmaMar) FDA-approved for metastatic SCLC; doxorubicin generic. The LMS combination is investigational. · **Geographic scope:** US + EU trial footprint; closed to accrual · **Verified:** 2026-08-12

Lurbinectedin is FDA-approved for small cell lung cancer, so off-label prescribing is technically possible, but the sarcoma combination is trial-defined and its first-line LMS trial (SaLuDo) closed to accrual before her diagnosis was corrected. There is no enrollment path and no published efficacy result yet to anchor an off-label payer request in LMS. The realistic posture is to watch the SaLuDo readout, which will bear directly on how the LMS-04 backbone evolves.

Next steps

1. Log SaLuDo (NCT06088290) as a watch item alongside the olaparib randomized trial; its result could reshape her first-line class at relapse or inform re-treatment decisions.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06088290		active not recruiting	yes	—

Manufacturer / sponsor contact

- **Company:** Jazz Pharmaceuticals (US marketer); originator PharmaMar
- **Med info phone:** 1-800-520-5568
- **Notes:** Phone verbatim from the current Zepzelca FDA label.

Payer / coverage: No LMS publication exists yet, so an off-label combination request would have nothing to stand on; revisit after SaLuDo reads out.

17. Olaparib + temozolomide

Aliases: olaparib, AZD2281, Lynparza, temozolomide, Temodar

Modality: small_molecule · **Regulatory:** Olaparib FDA-approved in BRCA/HRR-defined indications (ovarian, breast, pancreatic, prostate); temozolomide FDA-approved (glioma) and generic. The uLMS combination is off-label for both. · **Geographic scope:** US-wide for off-label prescribing; both dedicated trials US-based but closed to accrual · **Verified:** 2026-08-12

Both drugs are marketed (olaparib branded, temozolomide generic), so this combination is reachable off-trial by ordinary prescribing even though both dedicated uLMS trials are closed to new patients: NCI 10250 and the randomized successor NCT05432791 are each active-not-recruiting as of this check. The practical path at progression is an off-label prescription with a payer appeal built on the published phase 2 (ORR 27%, mPFS 6.9 months in pretreated uLMS), while watching the randomized trial for readout or reopening. The HRR and RAD51

story decides how hard to push for it.

Next steps

- 1. Send the somatic HRR panel and the germline hereditary-cancer panel now, off the 2024 archival block and a blood draw; results gate how hard this branch gets pushed.
- 2. On the RAD51 functional assay: re-verified this run, and there is still no CLIA laboratory offering it as an orderable catalogue test. Treat it as trial-linked or academic-collaboration only, and do not hold up the HRR panel waiting for it.
- 3. Set a recurring check on NCT05432791 for reopening or readout.
- 4. If progression arrives before trial access exists, file the off-label olaparib prior authorization with the JCO phase 2 attached, via AstraZeneca medical information (1-800-236-9933) for label support documents.
- 5. Ask whichever sarcoma center she lands at whether they run RAD51 foci in-house alongside a PARP trial.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03820019		active not recruiting	yes	—
NCT05432791		active not recruiting	yes	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca Pharmaceuticals LP (olaparib); temozolomide multi-source generic
- **Med info phone:** 1-800-236-9933
- **Notes:** Phone verbatim from the current Lynparza FDA label ('contact AstraZeneca at 1-800-236-9933').

Payer / coverage: Temozolomide is generic and cheap; olaparib is the coverage question. An off-label olaparib request in uLMS should lead with PMID 37467452, note the ongoing NCI-sponsored randomized phase 2/3 (NCT05432791), and attach the tumor HRR panel result if it shows BRCA2 or another HRR alteration, because a documented HRD state moves the request from speculative to biomarker-anchored. Expect an initial denial and plan the appeal.

Notes: Strongest non-cytotoxic signal in uLMS, and the one place where the access answer is genuinely nuanced: reachable off-trial today, but the smart sequence is biomarkers first, payer file second, trial watch throughout.

18. Pegylated liposomal doxorubicin (maintenance)

Aliases: pegylated liposomal doxorubicin, PLD, Doxil, Caelyx

Modality: small_molecule · **Regulatory:** FDA-approved (ovarian cancer, Kaposi sarcoma, multiple myeloma) and generic; STS maintenance use is off-label and investigational. · **Geographic scope:** Drug US-wide; the maintenance trial is Taiwan-only · **Verified:** 2026-08-12

PLD is a marketed generic any US center can give, so access is trivial; the actual question it represents, whether to maintain after anthracycline induction, is already answered inside her planned regimen by trabectedin maintenance. The randomized test of PLD maintenance (MELODY) is Taiwan-only, so it is evidence context rather than a destination. Worth knowing: that space runs on an 8-week post-anthracycline randomization window, the

kind of deadline maintenance decisions attract.

Next steps

1. No action; the LMS-04 design already assigns her maintenance drug. Revisit only if trabectedin maintenance proves intolerable.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06921637		recruiting	likely	Hui-Jen Tsai, MD; hjtsai@nhri.edu.tw ; +886-6-7000123

Payer / coverage: Generic PLD raises no coverage issues if her team ever wanted it; the clinical justification off-trial is the weak point, not the payment.

19. Pembrolizumab

Aliases: pembrolizumab, MK-3475, Keytruda

Modality: monoclonal_antibody · **Regulatory:** FDA-approved, including tumor-agnostic indications for TMB-H (≥ 10 mut/Mb) and MSI-H/dMMR solid tumors after prior therapy. uLMS use without one of those biomarkers is off-label. · **Geographic scope:** US-wide · **Verified:** 2026-08-12

Marketed everywhere; the question is not whether she can get pembrolizumab but under what banner. If the queued TMB and MMR testing comes back TMB-high (on a validated CGP platform; KEYNOTE-158 eligibility ran on the Foundation Medicine readout) or dMMR/MSI-high, the tumor-agnostic approvals make it label-available after progression on prior therapy, and coverage follows. Without one of those results, sarcoma use is off-label with weak histology-level evidence, since LMS sits in the immune-low microenvironment classes.

Next steps

1. Close the cheap gates: MMR four-stain IHC plus TMB and MSI from the comprehensive genomic panel already queued; specify a platform with validated TMB reporting.
2. Add PD-L1 with CD3/CD8 on the 2026 lung core for context; informative in sarcoma, not gating.
3. If TMB-H or dMMR returns, revisit at first progression, where the tumor-agnostic label applies.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02628067		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme LLC
- **Med info phone:** 1-877-888-4231
- **Product info:** <https://www.keytruda.com>

- **Notes:** Phone verbatim from the current FDA label ('contact Merck Sharp & Dohme LLC at 1-877-888-4231').

Payer / coverage: With a documented TMB-H or dMMR/MSI-H result on a validated platform, coverage is on-label and routine after progression on prior therapy. Without it, expect denial and a thin appeal; the biomarker is the whole access story here. A dMMR result would also open dostarlimab under the same tumor-agnostic logic.

20. Trabectedin + olaparib (preclinical combination)

Aliases: trabectedin, Yondelis, olaparib, Lynparza

Modality: small_molecule · **Regulatory:** Each component FDA-approved separately; the combination has no clinical development program in LMS. · **Geographic scope:** US-wide in theory; no protocol exists · **Verified:** 2026-08-12

Both drugs are marketed, so nothing physically prevents a prescriber from combining them, but no clinical trial has tested the pairing in LMS and the supporting evidence is one preclinical synergy study. Off-label access exists on paper; using it would mean treating outside any human safety experience with two myelosuppressive agents. This row exists to say the door is technically open and should not be walked through outside a protocol.

Next steps

1. Treat as a research idea, not an access pathway. If a protocol testing PARP-plus-trabectedin ever opens in sarcoma, the trial screener should catch it on a re-run.

Payer / coverage: No payer will cover the pairing as such, and there is no publication to anchor an appeal; coverage would ride on each drug's separate justification.

Notes: PARP1 expression, the proposed synergy predictor, has never been measured in this patient.

21. Trastuzumab deruxtecan (T-DXd)

Aliases: trastuzumab deruxtecan, T-DXd, DS-8201, Enhertu

Modality: ADC · **Regulatory:** FDA-approved (Enhertu, Daiichi Sankyo/AstraZeneca), including tumor-agnostic accelerated approval for HER2 IHC 3+ solid tumors after prior systemic therapy. · **Geographic scope:** US-wide · **Verified:** 2026-08-12

Marketed, and the tumor-agnostic accelerated approval for HER2 IHC 3+ solid tumors after prior therapy means one slide could make this label-available; HER2 has simply never been stained on her tumor and meaningful expression in uLMS is uncommon. The already-planned IHC bundle closes the question for the cost of one stain. Two case-specific cautions if it ever came into play: the drug's ILD risk against her bilateral infiltrative lung disease, and an 8,500 ft oximetry baseline that would make pneumonitis surveillance genuinely hard to read.

Next steps

1. Add HER2 IHC to the planned stain bundle on the archival block; a 3+ result changes the option set, anything

less closes the row.

2. If ever used, build pneumonitis surveillance around her altitude-adjusted baseline oximetry, documented before cycle 1.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04422309		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Daiichi Sankyo, Inc. (with AstraZeneca)
- **Med info phone:** 1-877-437-7763
- **Notes:** Phone verbatim from the current Enhertu FDA label.

Payer / coverage: With a documented IHC 3+ result and prior therapy, tumor-agnostic coverage applies. IHC 2+ or lower leaves only an unsupported off-label request in sarcoma.

Clinical trial (10)

22. ATR inhibition (ATRX/ALT-directed hypothesis)

Aliases: ATR inhibitor, ceralasertib, berzosertib, elimusertib

Modality: small_molecule · **Regulatory:** No ATR inhibitor holds regulatory approval; class-wide investigational.
· **Geographic scope:** Nothing currently open that fits · **Verified:** 2026-08-12

No ATR inhibitor is approved anywhere, so trial enrollment would be the only route, and no ALT- or ATRX-selected ATR trial in sarcoma was surfaced by this dossier; there is nothing to enroll in today. The scientific case is also contested: the founding claim that ALT status predicts ATR-inhibitor sensitivity failed replication, and the sarcoma cell-line data point to telomere length instead. This stays a watch item pending the ATRX IHC result, not an access pathway.

Next steps

1. Run the ATRX IHC in the planned stain bundle anyway; it is cheap, ALT-concordant in uLMS, and informs biology beyond this one hypothesis.
2. Flag ATR-inhibitor sarcoma protocols for the trial screener at future re-runs rather than tracking manually.

Payer / coverage: Not applicable; no product and no protocol.

23. Anlotinib (catequentinib)

Aliases: anlotinib, catequentinib, AL3818

Modality: small_molecule · **Regulatory:** Approved in China (NMPA) for advanced soft tissue sarcoma settings; investigational in the US, no FDA filing approved. · **Geographic scope:** Marketed in mainland China only; US access via APROMISS indication E at best · **Verified:** 2026-08-12

Approved and marketed only in China, where it holds NMPA soft tissue sarcoma indications; there is no US or EU approval, so no off-label path exists for a US patient. The US-inclusive APROMISS phase 3 shows as recruiting on the registry, but both LMS-specific indications are closed or suspended to new accrual and only the loosely defined any-sarcoma indication E remains open. The honest read: this class need is already served by label-available pazopanib and the zanzalintinib trial.

Next steps

1. Treat as low priority; if the TKI class is wanted, pazopanib is on label and zanzalintinib has an open LMS cohort.
2. If still curious, email Shiyings@advenchen.com to ask whether indication E is accepting LMS patients at a US site.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03036819		recruiting	unconfirmed	Shiyong Clinical Trial Manager; Shiyings@advenchen.com ; 8055301550

Manufacturer / sponsor contact

- **Company:** Advenchen Laboratories (US development); Chia Tai Tianqing (China marketer)
- **Notes:** No US medical-information line; the Advenchen trial managers above are the published contacts.

Payer / coverage: Not obtainable through any US payer; the China marketing authorization does not travel.

24. B7-H3 ADCs (MGC026; GSK5764227/HS-20093)

Aliases: MGC026, GSK5764227, HS-20093, GSK'227, B7-H3, CD276

Modality: ADC · **Regulatory:** Both agents investigational; no approvals. The class's prior entrant vobramitamab duocarmazine was halted after safety findings. · **Geographic scope:** US sites for MGC026 and EMBOLD; ARTEMIS-002 mainland China only · **Verified:** 2026-08-12

No B7-H3 ADC is approved anywhere, so trials are the only route, and two US-recruiting phase 1 programs are open: MacroGenics' MGC026, which names sarcoma on its tumor list, and GSK's EMBOLD study of GSK5764227, an all-comers escalation whose sarcoma rationale comes from Chinese sister studies. Both expect exhausted standard options, placing them after LMS-04 and probably one further line. Neither gates on B7-H3 IHC, but the planned local stain still earns its cost by telling the board whether a referral is worth the trip; 97% of soft tissue sarcomas stain positive.

Next steps

1. Add B7-H3/CD276 IHC to the planned stain bundle; a strong positive is the single most likely surface-target finding in this histology.
2. After standard options are exhausted, contact MacroGenics (info@macrogenics.com, 301-251-5172) about the MGC026 sarcoma cohort and GSK (877-379-3718) about EMBOLD slots, comparing which open site is actually reachable for her.
3. Get her ECOG documented; EMBOLD's 0-1 ceiling is strict.
4. Skip ARTEMIS-002 unless circumstances put her in China.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06242470		recruiting	yes	Global Trial Manager; info@macrogenics.com; 301-251-5172
NCT06551142		recruiting	likely	US GSK Clinical Trials Call Center; GSKClinicalSupportHD@gsk.com; 877-379-3718
NCT05820123		recruiting	likely	Wei Guo, MD; bonetumor@163.com; 010-88326656
NCT05521117		terminated	no	—

Manufacturer / sponsor contact

- **Company:** MacroGenics, Inc. (MGC026); GlaxoSmithKline / Hansoh (GSK5764227)
- **Med info phone:** 301-251-5172
- **Med info email:** info@macrogenics.com
- **Compassionate / expanded access:** <https://www.macrogenics.com/expanded-access/>
- **Notes:** MacroGenics' posted policy considers expanded access case by case only for products in phase 2 or 3; MGC026 is phase 1, so trial enrollment is the only route today and the policy page lists no dedicated contact.

Payer / coverage: Trial-supplied agents. The B7-H3 IHC stain itself is a routine pathology order on the archival block.

25. Brenetafusp (PRAME x CD3 ImmTAC)

Aliases: brenetafusp, IMC-F106C

Modality: bispecific_other · **Regulatory:** Investigational (Immunocore); phase 3 in melanoma, no approval.

Geographic scope: US (NCI network); not yet open, histology-excluded for now · **Verified:** 2026-08-12

Investigational and, for her histology, not yet reachable: the NCI rare-cancers phase 2 that brings this drug into sarcoma names synovial sarcoma and myxoid round cell liposarcoma only, and it has not opened recruitment. The row matters as a watch item because it is the off-the-shelf alternative to PRAME TCR-T on the identical HLA-A*02:01 eligibility platform, and histology expansion is plausible as the program matures.

Next steps

1. The HLA typing drawn for the IMA203 row covers this class too; no additional test needed.
2. Re-screen this program at each pipeline re-run for LMS-admissible cohorts.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07626367		not yet recruiting	no	—

Payer / coverage: Not applicable while no admissible protocol exists.

26. FAP-targeted radioligands ([225Ac]RTX-2358; [177Lu]Lu-FAP-2286)

Aliases: [225Ac]RTX-2358, RTX-2358, [64Cu]LNTH-1363S, [177Lu]Lu-FAP-2286, FAP-2286

Modality: radioligand · **Regulatory:** Both investigational; no approvals. RTX-2358 is a Ratio Therapeutics alpha-therapy program, FAP-2286 a Novartis beta-emitter theranostic. · **Geographic scope:** US + Canada; RTX-2358 at 5 US sites, LuMIERE at 20 US sites · **Verified:** 2026-08-12

Neither agent is approved, so both run through trials, and both pass the door-openness test for a US patient: the alpha-emitter RTX-2358 phase 1 is recruiting sarcoma specifically at five US sites plus Toronto, and Novartis' LuMIERE beta-emitter study is recruiting at 20 US sites with sarcoma not excluded. Selection in each is a FAP PET scan done at screening, so no local FAP testing is needed to knock on the door. One prior regimen suffices for RTX-2358, so she could screen directly after LMS-04; both trials also require ECOG 0-1, which is undocumented for her.

Next steps

1. No FAP testing needed in advance; the screening PET is the gate and the trial provides it.
2. After LMS-04 progression, call the nearest RTX-2358 site from the list above (five US options spanning both coasts, Texas, Minnesota and Ohio) and ask about screening-slot timing.
3. Keep LuMIERE (Novartis, 1-888-689-6682) as the beta-emitter fallback a line later.
4. Document ECOG and review the radiation-adjacent exclusions (prior RT fields) against her record before screening.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07136565		recruiting	likely	—
NCT04931210		recruiting	unconfirmed	Novartis Pharmaceuticals; novartis.email@novartis.com; 1-888-689-6682

Manufacturer / sponsor contact

- **Company:** Ratio Therapeutics (RTX-2358); Novartis (FAP-2286)
- **Med info phone:** 1-888-689-6682

- **Med info email:**novartis.email@novartis.com
- **Notes:** Novartis contact verbatim from the LuMIERE registry record; Ratio Therapeutics publishes no medical-information line, so the trial site contacts are the channel.

Payer / coverage: Trial-supplied isotopes and screening PETs. Radioligand programs run at nuclear-medicine-capable centers only, which compounds the travel question for a patient whose tolerance for travel was never stated.

Notes: Crossfire physics makes this class attractive in a histology where surface-antigen heterogeneity undermines ADC cutoffs, and the marrow-stroma FAP toxicity signal from CAR-T work is the caution the preclinical file carries against it.

27. IMA203 / IMA203CD8 (PRAME TCR-T)

Aliases: IMA203, IMA203CD8, ACTengine IMA203

Modality: other · **Regulatory:** Investigational (Immatix); phase 3 development in melanoma, no approval. · **Geographic scope:** US: 13 sites in 10 states; also Germany · **Verified:** 2026-08-12

Investigational cell therapy; the ACTengine basket is the access route and it runs on biology rather than histology: HLA-A*02:01 genotype plus PRAME expression by the sponsor’s assay, with sarcomas treated in earlier cohorts, so uLMS is not categorically excluded the way the MAGE-A4 programs exclude it. Both gates are unmeasured in her; an HLA typing blood draw settles half the question for the whole engineered-T-cell class in one step. Thirteen US sites, including one in Colorado.

Next steps

1. Draw HLA class I allele-level typing now, on the same visit as the germline panel; it is a one-time result that gates this entire class.
2. If HLA-A*02:01-positive, email ctgovinquiries@immatix.com after LMS-04 progression to ask about PRAME screening of her tissue and current sarcoma slot policy.
3. Confirm documented ECOG 0-1 before screening.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03688124	1/2	recruiting	unconfirmed	Immatix US, Inc.; ctgovinquiries@immatix.com ; +1 346 204-5400

Manufacturer / sponsor contact

- **Company:** Immatix Biotechnologies / Immatix US, Inc.
- **Med info email:**ctgovinquiries@immatix.com
- **Notes:** Contact verbatim from the registry central-contact block; no separate medical-information line pre-approval.

Payer / coverage: Trial-supplied therapy; cell-therapy trials concentrate at large centers and involve an

apheresis-to-infusion timeline the family should understand before committing.

28. Ivonescimab (PD-1 x VEGF bispecific)

Aliases: ivonescimab, AK112, SMT112

Modality: bispecific_other · **Regulatory:** Investigational in the US; no FDA approval. · **Geographic scope:** US: MSK network only (New York, NY and NJ regional sites) · **Verified:** 2026-08-12

No FDA approval, so the MSK single-arm phase 2 is the only US route. The protocol enrolls LMS after one to three prior lines and carries an unusual clause admitting patients who decline first-line standard care, so it is technically open to her today; the sounder sequence is LMS-04 first, which keeps her eligible here afterward. All seven sites are MSK locations in New York and New Jersey, a long way from a mountain-state residence, and her travel tolerance was never actually stated.

Next steps

1. Hold until the LMS-04 outcome declares itself; she remains eligible with 1-3 prior lines.
2. At that point email the MSK sarcoma trials inbox (zzPDL_MED_Sarcoma_Clinical_Trials@mskcc.org) referencing NCT07516925.
3. Raise the travel question directly with her before any New York screening visit is booked.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07516925		recruiting	yes	Sandra D'Angelo, MD; 646-888-4159

Payer / coverage: Trial-supplied drug at an investigator-sponsored MSK study; routine-care costs bill to insurance as usual.

29. Pulmonary suffusion + metastasectomy

Aliases: pulmonary suffusion

Modality: other · **Regulatory:** Investigational surgical-regional technique under a single-center IND protocol. · **Geographic scope:** US: single site (Buffalo, NY) · **Verified:** 2026-08-12

An experimental regional-chemotherapy technique delivered only inside the Roswell Park phase 1/2, so trial enrollment in Buffalo is the sole route. It sits at the same decision point as standard metastasectomy, after induction produces operable anatomy, and shares all the same fitness caveats plus a single-site travel commitment. A live US protocol at exactly the consolidation branch her oncologist left open, but one that only matters if surgery becomes plausible at all.

Next steps

1. Park until a post-induction scan shows metastasectomy-appropriate anatomy.
2. If it does, email Kenneth.Seastedt@Roswellpark.org to compare suffusion-plus-resection against standard resection closer to home before committing to Buffalo.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT039615234		recruiting	unconfirmed	Kenneth Seastedt; Kenneth.Seastedt@Roswellpark.org ; 716-845-2300

Payer / coverage: Trial covers the investigational component; the metastasectomy itself bills as standard surgery.

30. REC-617 (selective CDK7 inhibitor)

Aliases: REC-617

Modality: small_molecule · **Regulatory:** Investigational; no regulatory approval or filing. Phase 1 dose finding. ·

Geographic scope: US: single site (Houston, TX) · **Verified:** 2026-08-12

Investigational with no approval anywhere, so the MD Anderson phase 1b is the only route. It wants the mirror image of the abemaciclib trial: RB1-null LMS by IHC (0% expression), at least one prior line, a biopsy-amenable lesion, and measurable disease, which slots it after LMS-04 if her RB1 stain comes back null. Given near-universal RB1 loss in LMS genomics, this is statistically the likelier of the two CDK doors, though the CDK7 mechanistic case in LMS itself is thin.

Next steps

1. Same first move as the abemaciclib row: RB1 IHC on the archival block.
2. If RB1-null, email Elise Nassif (efnassif@mdanderson.org, 281-460-0607) with the diagnosis, line of therapy, and stain result to ask about slot timing.
3. Confirm a biopsy-amenable lesion exists; the protocol requires one.
4. Document her real ECOG before screening.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07633756		recruiting	unconfirmed	Elise Nassif, MD; efnassif@mdanderson.org ; 281-460-0607

Manufacturer / sponsor contact

- **Company:** Recursion Pharmaceuticals (agent); trial run at MD Anderson
- **Notes:** No published medical-information line for this investigational agent; the trial contact is the working

channel.

Payer / coverage: Trial-supplied drug; standard-of-care costs (imaging, biopsies) bill as usual, worth confirming with the site's financial counselor.

31. Zanzalintinib (XL092)

Aliases: zanzalintinib, XL092

Modality: small_molecule · **Regulatory:** Investigational (Exelixis); no approval in any indication. · **Geographic scope:** US: single site (Chicago, IL) · **Verified:** 2026-08-12

Investigational successor-generation TKI with no approval; the Northwestern phase 2 has a dedicated LMS cohort requiring at least two prior lines, so it sits third-line or later for her, after LMS-04 and one further regimen. Pathology review at an NCCN center is required, which her completed reference-laboratory re-review should satisfy. Single site in Chicago.

Next steps

1. Park until the third-line decision point.
2. If reached, email cancer@northwestern.edu referencing NCT06571734 and cohort 1, with the reference-pathology report attached.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06571734		recruiting	yes	Coordinator; cancer@northwestern.edu ; 3126951301

Manufacturer / sponsor contact

- **Company:** Exelixis, Inc.
- **Notes:** No medical-information line applicable pre-approval; the trial coordinator address is the working contact.

Payer / coverage: Trial-supplied drug. The approved comparator at the same decision point is label-covered pazopanib, which is the practical alternative if travel to Chicago does not suit.

Unavailable (1)

32. Doxorubicin + olaratumab

Aliases: olaratumab, LY3012207, Lartruvo, doxorubicin

Modality: monoclonal_antibody · **Regulatory:** Approval withdrawn; olaratumab is no longer marketed following

the negative ANNOUNCE confirmatory trial. · **Geographic scope:** Withdrawn worldwide · **Verified:** 2026-08-12

Not obtainable, and that is the point of the row: olaratumab's accelerated approval in soft tissue sarcoma was withdrawn after the confirmatory ANNOUNCE trial showed no survival benefit, and Lilly took the drug off the market. No successor program exists. The dossier kept ANNOUNCE only for its doxorubicin control-arm benchmark (median OS 21.9 months in the LMS stratum with modern supportive care).

Next steps

1. None; recorded so the answer to 'could we add olaratumab' is visibly no rather than silently missing.

Manufacturer / sponsor contact

- **Company:** Eli Lilly and Company
 - **Notes:** Product discontinued; no access program exists.
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